



Contents lists available at ScienceDirect

International Emergency Nursing

journal homepage: www.elsevier.com/locate/aaen

Workplace bullying in emergency nursing: Development of a grounded theory using situational analysis

Lisa A. Wolf^{a,*}, Cydne Perhats^a, Paul R. Clark^{b,2}, Michael D. Moon^{c,3},
Kathleen Evanovich Zavotsky^d

^a Institute for Emergency Nursing Research, Emergency Nurses Association, Des Plaines, IL, United States

^b University of Louisville School of Nursing, Norton Healthcare Institute for Nursing, Kentuckiana Chapter, Louisville, KY, United States

^c University of the Incarnate Word, San Antonio Chapter, San Antonio, TX, United States

^d Center for Professional Development, Innovation and Research, Robert Wood Johnson University Hospital, United States

ARTICLE INFO

Keywords:

Emergency nursing
Workplace bullying
Lateral violence
Situational analysis

ABSTRACT

Background: The Institute of Medicine recognizes that the workplace environment is a crucial factor in the ability of nurses to provide safe and effective care, and thus interactions that affect the quality and safety of the work environment require exploration.

Objectives: The purpose of this study was to use situational analysis to develop a grounded theory of workplace bullying as it manifests specifically in the emergency care setting.

Methods: This study used a grounded theory methodology called situational analysis. 44 emergency RNs were recruited to participate in one of 4 focus group sessions, which were transcribed in their entirety, and, along with field notes, served as the dataset.

Results: This grounded theory describes the characteristics of human actors and their reactions to conditions in the practice environment that lead to greater or lesser levels of bullying, and the responses to bullying as it occurs in U.S. emergency departments.

Discussion: Workplace bullying is a significant factor in the dynamics of patient care, nursing work culture, and nursing retention. The impact on patient care cannot be overestimated, both in terms of errors, substandard care, and the negative effects of high turnover of experienced RNs who leave, compounded by the inexperience of newly hired RNs. An assessment of hospital work environments should include nurse perceptions of workplace bullying, and interventions should focus on effective managerial processes for handling workplace bullying. Future research should include testing of the theoretical coherence of the model, and the testing of bullying interventions to determine the effect on workplace environment, nursing intent to leave/retention, and patient outcomes.

1. Introduction

The Institute of Medicine (IOM) recognizes that the workplace environment is a crucial factor in the ability of registered nurses (RNs) to provide safe and effective care [1]. Dysfunctional work environments in which bullying occurs can have a significant impact on the ability of RNs to safely and effectively care for patients.

Investigation of a complex social phenomenon requires an examination of multiple factors. This led to the selection of Clarke's [2] situational analysis model, which extends and expands the grounded

theory methodology that is based on a constructivist, interpretive paradigm [2–4]. Clarke uses three main types of mapping to analyze data: *situational maps* that lay out the main human and nonhuman elements and examine relationships between them; *social worlds maps* that describe the actors, key nonhuman elements and the areas of commitment within which they engage and help to interpret situations, and *positional maps* that describe the major positions taken (and not taken) about complicated issues in the phenomenon of interest [2].

* Corresponding author at: 915 Lee Street, Des Plaines, IL 60016, United States (Work). 110 Middle Street, Hadley, MA 01035, United States (Home).

E-mail addresses: lwolf@ena.org, noblewolf3@aol.com (L.A. Wolf).

¹ Pioneer Valley Chapter.

² Kentuckiana Chapter.

³ San Antonio Chapter.

<http://dx.doi.org/10.1016/j.ienj.2017.09.002>

Received 29 June 2017; Received in revised form 31 August 2017; Accepted 3 September 2017

1755-599X/ © 2017 Elsevier Ltd. All rights reserved.

2. Literature review

Workplace bullying occurs when employees (i.e., the targets of bullying) face prolonged exposure to negative behaviors against which they feel unable to defend themselves [5]. For the purposes of our research inquiry, we used the terms lateral/horizontal violence, bullying, and incivility/aggression interchangeably (despite some differences in meaning); however, for simplicity we've chosen to exclusively use the term workplace bullying.

Research findings suggest that bullying frequently occurs among RNs [6] to the extent that up to 40% of nurses report an intent to leave because of it [7]; this is significant especially for new nurses, in that new RNs reported being bullied at work which resulted in 30% of new RNs leaving their jobs within the first year [8] and 57% by the second year [9].

Bullying is associated with emotional exhaustion, poor mental health [10], psychological distress symptoms [11], job dissatisfaction, absenteeism, intention to leave, and high turnover rates [12–16] and acts as a trigger for lateral violence [17]. Workplace bullying is described as both a cause and an effect of physical and emotional fatigue [18] and is reported as an ordinary and too often tolerated part of nursing socialization [18–21]. Bullying was also identified as a barrier to addressing workplace violence originating from patients and visitors [22], and potentially affects overall health and safety of the entire workplace [23,24].

Bowie [25] describes four types of violence that exist both independently, and concurrently. Types 1 and 2 involve perpetrators having no connection to the workplace (*Intrusive violence*) and patients committing acts of violence on nurses (*Consumer violence*), respectively. Types 3 and 4 have more relevance to this study. Type 3 (*Relationship violence*) involves aggressive acts by persons who have an employment based relationship with an organization, which could include an employee's current or former spouse, family or significant other. Type 3 violence could also include cases of stalking or former employees seeking justice for perceived previous wrongs as well as workplace bullying and harassment. Type 4 (*Organizational violence*) involves organizations knowingly placing their workers in dangerous or violent situations or allowing a climate of bullying or harassment to thrive in the workplace. This becomes important when nurses who are victims of type 3 violence attempt to seek help from administration and experience type 4 violence when they fail to receive assistance and/or are further bullied.

2.1. Purpose

The purpose of this study was to explore and develop a substantive theory of workplace bullying as it manifests specifically in the emergency department (ED) setting, using a modified, grounded theory approach.

3. Methods

This study used a methodology called *situational analysis* as described by Clarke [2]. Clark's situational analysis model uses maps derived from qualitative data to describe the type and relationship of elements that make up the phenomenon of interest. Situational maps lay out the main human and nonhuman elements and examine relationships between them; social worlds maps describe the actors, key nonhuman elements and the areas of commitment within which they engage and help to interpret situations at the organizational level, and positional maps describe the major positions taken (and not taken) about complicated issues in the phenomenon of interest. This approach allowed for rich, thick descriptions that lend themselves to the development of a substantive theory of workplace bullying in the ED and to interventional strategies. Furthermore, it provides the reader context to understand the complexities involved with this social phenomenon as it

Table 1
Participants' demographics (N = 43).

Characteristic		Participants (%)			
Gender	Female	86.0			
	Male	14.0			
Age	25–34	4.7			
	35–44	23.3			
	45–54	34.9			
	55–64	32.6			
	> 64	4.7			
Highest Educational Degree in Nursing	Nursing Diploma	2.3			
	Associate	18.6			
	Bachelor	55.8			
	Master's	20.9			
	Doctorate	2.3			
Primary ED Role	Case Manager	2.3			
	Charge Nurse	9.3			
	Clinical/Nurse Educator	9.3			
	Director	9.3			
	Manager	16.3			
	Staff Nurse	51.2			
	Other	2.3			
ENA Member	Yes	95.3			
	No	4.7			
Years of Experience		Mean	SD	Min	Max
	As a nurse in all areas, including the ED	22.2	14.0	1.0	44.0
	As an emergency nurse only	17.7		1.9	
	In current ED	8.5	11.3	0.4	43.0
	In all areas of emergency care, excluding nursing (e.g., tech)	8.0	7.8	0.0	29.4
			10.7		40.1

is related to the ED and as the relevant literature is presented; the depth and breadth of this approach helps to establish the credibility, transferability, dependability, and confirmability of this study.

3.1. Sample

A purposive, convenience sample of 43 emergency RNs was recruited through email and social media from a list of registered attendees at the Emergency Nurses Association's Conference in Los Angeles, CA (Table 1). To ensure sampling representation by gender, geographic location, and work role, both staff nurses and those who were in administrative positions were recruited (Table 2).

Four separate focus groups were held at EN2016, with attendees separated into two employee categories, one consisting of staff RNs (N = 28), and the other of managers, directors, and educators (N = 15). Prior to implementation of the study, we obtained Institutional Review Board (IRB) approval from the Chesapeake Institutional Review Board (Pro00018760) and a Certificate of Confidentiality (CoC) from the National Institutes of Health (NIH). No compensation or incentive was offered to participants who completed the demographic survey and/or participated in the focus groups.

3.2. Data collection

Demographic information was collected about individual participants and the practice settings in which they worked (Table 1), using online survey software (Qualtrics, Provo, UT). All the members of the research team are fulltime researchers or faculty who may have been known to the participants by name. The purpose of the study was explained to participants before the start of each one-hour focus group session. Focus groups took place at the EN2016 conference, and were

Table 2
Facility characteristics (N = 43).

Characteristic	Participants (%)
ED Patient Population	
General ED	86.0
Adult Only ED	9.3
Pediatric Only ED	4.7
Facility Type	
Non-Government, Not-For-Profit	65.1
Investor-Owned, For-Profit	7.0
State or Local Government	23.3
Federal, Military, or VA	4.7
Geographic Location	
Urban	46.5
Suburban	30.2
Rural	23.3
Annual ED Patient Visits	
10,001–20,000	11.6
20,001–30,000	18.6
30,001–40,000	7.0
40,001–50,000	16.3
50,001–75,000	16.3
75,001–100,000	11.6
> 100,000	14.0
Did not know	4.7
Geographic Distribution	
Outside the U.S.	7.0
Northeast	6.9
Midwest	13.9
South	41.8
West	30.1

facilitated by the Principal Investigator (LW) and two of the three other researchers (KEZ, MDM, and PRC), who took field notes on a common data collection form. Each focus group was audiotaped. A fourth researcher (CP) also took field notes and maintained records for each session. To determine data saturation, the research team held a debriefing session after each focus group to clarify specific questions. The focus group discussions were transcribed in their entirety and, along with the collected field notes, served as the dataset. A semi-structured guide was employed, using the following questions:

Focus Group Interview Questions

- 11 What are emergency RNs' perceptions of workplace bullying in their ED?
- 21 What are emergency RNs' perceptions of the components of workplace bullying?
- 31 What are emergency RNs' perceptions of patient and RN outcomes of workplace bullying?
- 41 What actions do emergency RNs take when they have concerns that they or their colleagues are being bullied?

3.3. Data analysis

Demographic data were exported to an SPSS (Version 24.0) database and descriptive analyses were performed. The transcribed focus group data were analyzed using mapping techniques derived from Clarke's situational analysis process [2]. The five members of the research team developed situational maps, social world maps, and positional maps during data coding and analyses and identified *sites of silence* in our data. Clarke [2] states that sites of silence occur when researchers suspect that there are underlying dynamics that have not appeared explicitly in the data. As the Principal Investigator and three of the other researchers were all experienced emergency nurses, this process assisted in identifying areas of bias. Each investigator analyzed the data separately and produced situational maps (Fig. 1) that were subsequently analyzed by the team with the incorporation of social world and positional maps to develop a consistent theoretical meaning. Veracity was established by external review of the theoretical model, by member-checking at the end of each focus group session, and again by allowing participants to review, confirm, and comment on or amend the

final model in response to an email query. All of the respondents (16 out of 43) confirmed our perceptions and descriptions of the phenomenon as depicted in Fig. 2.

4. Findings/theory generation

An overview of a grounded theory of workplace bullying.

4.1. Theoretical elements

Theoretical elements that comprise the model include *human and practice elements*, *the dominant narrative*, *dynamics of deliberate aggression and exclusion*, and *nurse and patient outcomes*. Each is described further and its position in the model explained. Supporting quotes are included from focus group participants, each identified by a unique number and his or her gender and primary role in the emergency department, as in the following examples: NF1 Staff Nurse = nurse female #1, a Staff Nurse; NF14 ED Director = nurse female #14, an ED Director.

4.2. Situational maps: human and practice elements and their relationships (Fig. 2)

Study participants identified a variety of elements that impact the quality and quantity of bullying in the ED workplace including nurses and other ED staff, charge RNs, ED managers, and patients. The charge RN role was identified as a prime driver for bullying in the presence of a cultural narrative permissive of workplace bullying.

[I] think it tends to be charge nurses and persons that are in a leadership role that over time have assumed this identity that they need to be aggressive because they shoulder responsibility, and when they're confronted with some of the bullying in my department, that's exactly what they tell me: "We have a really stressful job."... But that is actually what they say. They feel that they're so stressed, and they have so much responsibility that they have to carry this persona. (NMM3)

Group actors included the ED team (RNs (individually and in cliques), technicians, secretaries, physicians, and others who provide care to patients), the hospital leadership (ED directors and managers, the chief nursing officer, and human resources personnel who perpetuate conditions that were perceived as actively or passively contributing to a bullying culture).

... you'll see this power struggle... to push you, push that male nurse, to get them confrontational, and then if you do stand up and say something—stand up for yourself—then the charge nurse will lie to the manager or leadership, even to the point of making up stories to then get this pack together to, you know, get you out of there... (SNF23)

Components of the ED practice environment that contributed to bullying included nurses' emotional burden, a high-stress, chaotic workplace, and violent, aggressive, and entitled behavior from patients toward staff.

4.3. Filtered through the dominant narrative

Participants repeatedly discussed a dominant narrative that included some aspect of hazing, such as having to prove yourself, or operating from the cultural paradigm that nurses eat their young [26]. This finding is underscored in a study of Canadian nurses [27] in which the researchers reported that these paradigms are normative in nursing culture. Our participants reported that bullies in the dominant culture often justified their actions by noting that they were "toughening them up," or helping them "develop a thick skin," ostensibly to prepare them to provide better care, a paradox because it is inconsistent with the educational paradigm that competence builds confidence [28].

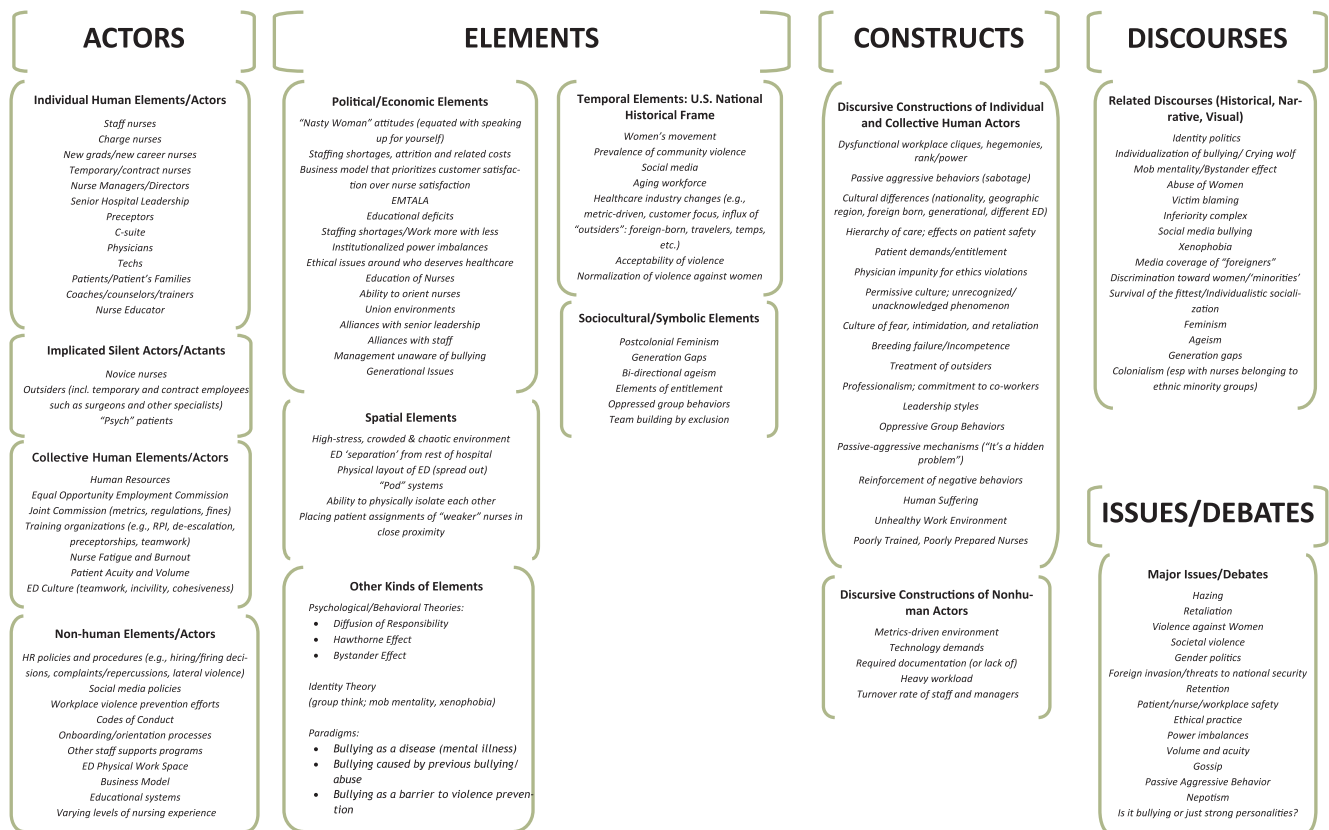


Fig. 1. Synthesized situational map identifying elements of the phenomenon.

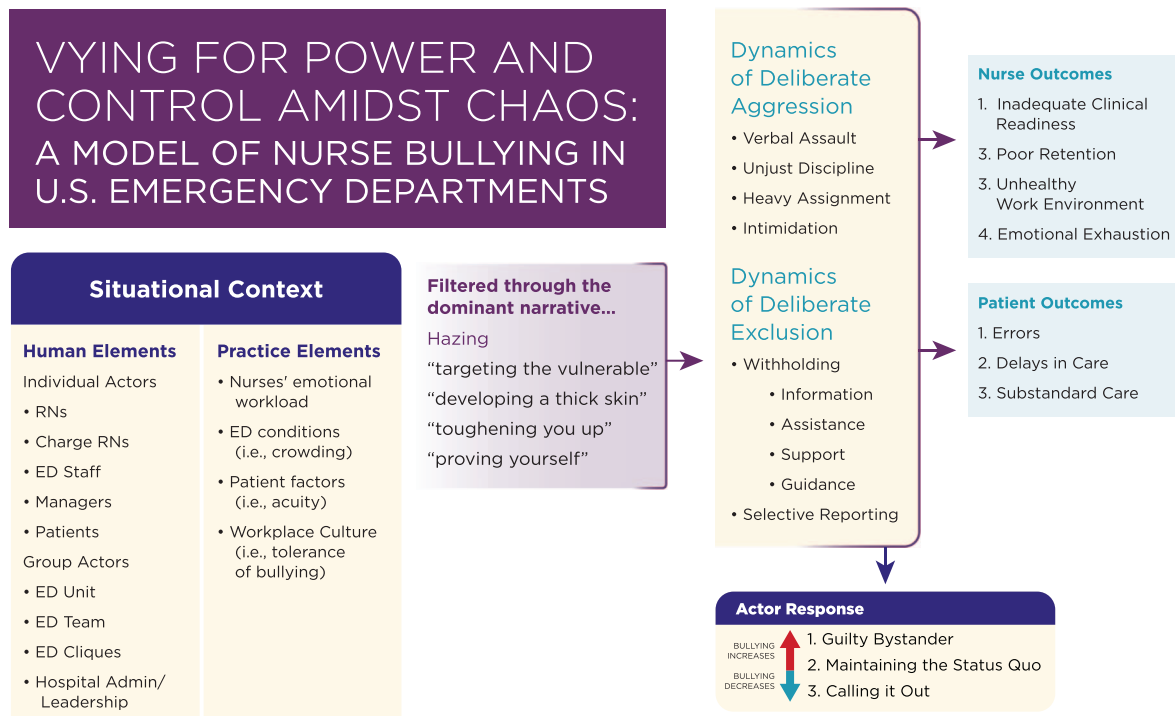


Fig. 2. Theoretical model of workplace bullying.

... you have to prove you're worthy of being a member of the team, and sometimes that involves a little bit of unfair assignment making or... kind of talking behind the nurse's back if they're in a room too long without really communicating with that person... So I think a

lot of gossiping, but also you have to prove yourself in order to be a worthy member of the team. (SNF21)

I feel like, as a newer nurse, there is a perception that it's just the way it's done, that, you know, there's a perception that the older

nurses eat their young and it's how they toughen you up and get you ready to work. (SNF38)

4.4. Social worlds map: actors, key nonhuman elements and areas of commitment

Dynamics of exclusion and aggression: The model shown in Fig. 2 depicts bullying in terms of the dynamics of deliberate exclusion and aggression. The dynamics of exclusion are marked by the withholding of information, assistance, support, and guidance which form the boundaries of safe patient care provision, creating groups among the ED staff that were either inside or outside of those boundaries. The dynamics of exclusion also include selective reporting – relaying information to managers and administrators highlighting negative nursing behavior and omitting context, breeding a culture of failure that leads to deficiencies in nursing practice and patient care.

The second dynamic was one of aggression, marked by false accusations, intimidation, and derogatory language leveled against RNs or the care they provide. The charge nurse role, whether provisional or full time, has a greater opportunity to exercise the dynamics of aggression, by manipulating nursing assignments in ways that could support or undermine RNs. Charge RNs can support staff nurses by giving them a patient volume and acuity level appropriate to their experience and expertise. Conversely, charge nurses can also set RNs up for failure by assigning too many patients, a rapid succession of patients, or high acuity patients that fall outside the experience level of the RN, making it difficult to provide safe and adequate care. Beyond charge RNs, anyone in a position of authority (e.g., preceptors, managers, physicians) can perpetuate this sort of bullying.

I think that bullying... extends into how you do assignments; it extends into how you distribute patients. I watched a nurse... [who] assigned an incoming stroke patient... to a psych nurse in the ER so that she could watch the psych nurse screw it up, so that she could come in and rescue the psych nurse and then try to get the psych nurse fired for missing a stroke.... But, at the end of the day, the charge nurse literally wrote up the psych nurse and tried to get her fired, and the only thing that saved [the psych nurse] was how hard the EMS pushed to save her job... (SNF35)

Participants described this type of bullying behavior as particularly insidious because it is marked by omission rather than commission and is very difficult to identify, call out, or discipline. Further complicating the identification and resolution of this type of behavior is the lack of documentation, which is a key element necessary for identifying acts of bullying.

It's hard to prove because there's nothing specific, like "Okay, what happened? Why didn't no one help this nurse when this is going on?" Well we—all of us—are busy with our patients. Okay, prove it: prove that no, they weren't, they just chose not to. (SNF17)

More overt and targeted bullying was perceived as occurring higher up in the administrative chain. Participants reported managers documenting problems about RNs that they wanted to get rid of, while ignoring the same problems about RNs who were a part of the *in-group bubble* (see discussion below).

And our problem is that our bullying goes all the way up—manager, director—and we see them all the time, but it's, I mean my director will walk around and be like, "Yeah, I want to get rid of that nurse."... I mean, even her co-director over in another department, she's like, "You work with her a lot, you need to keep a paper trail on her 'cause she's not doing a very good job and we need to prove it." And so when it's going that high up, what do you even do? (SNF31)

This type of bullying may occur as a function of systemic culture,

and thus typifies Type 4 (Organizational) violence as described by Bowie [25]. The power gradient between administration and staff causes a fear of retaliation, discourages reporting, perpetuates the bullying behavior, and stymies managers who seek solutions.

And what it was, they didn't have the facts. HR came and interviewed everybody in the department separately and realized that the bullies were fabricating stuff and they realized interviewing not just the person who were [sic] involved, but other people who kept quiet. Cuz that's what most people do: "They're not picking on me, I'm going to keep quiet over here." (SNF26)

Participants described the unit culture in which bullying occurs as toxic, and a prime reason for RNs leaving the ED. Participants did not describe all in-group members as bullies, but group status influenced access to information and help and the probability of experiencing retaliation. Dellasega [29] also identified nurse bully types that are consistent with those we have identified.

In-groups could be formal groupings (e.g., management or charge RNs) or informal groupings led by other staff. In-group membership had no objective rules, but seemed to be determined by group consensus, and negative behavior by in-group members was tolerated at levels not permitted to outsiders.

Out-groups could be comprised of RNs who were new graduates, new to that ED, foreign-born, or in some way "other". Participants who identified as "out-group" reported instances of targeting by bullies (of both themselves and colleagues), and described situations in which bullies withheld information, assistance, support, or guidance, or engaged in selective reporting.

My perspective is all hospitals are that way as well. If you have been there for a long time, you're in; if not, you're out. But I will say that over the years I've become a little tired of fishing new grads out of the bathroom crying because of the way that they've been treated at work. (NMF14)

We have a nurse in our ER, he's been a nurse for two years and in the ER almost one now, and a seasoned nurse... she says, "I'm not going to work with him," and I said, "Why not?"... "Well, he was on the phone the other day giving advice to a young mother about how much ibuprofen to give to her five-month old; you're not supposed to give a five-month old ibuprofen." Well, instead of saying that to [the RN], she just, you know, talked behind his back. He could have learned from that. (SNF21)

One participant captured the feelings that develop when nurses work on a unit where a culture of bullying exists:

... the culture of nursing in that facility was so awful... you went home every day beaten down and abused. (SNF38)

Nurse and patient outcomes: Our model describes the outcomes of workplace bullying: poor nurse retention, inadequate clinical readiness, and an unhealthy work environment, all of which affect the quality of patient care [11]. Participants reported that patient care was impeded by withholding information, inappropriate assignments to unseasoned nurses, inexperienced and poorly trained staff, and high staff turnover. An Australian study [30] reported similar examples of attacks on personal reputation, erosion of professional competence, and manipulation of work assignments.

I've seen three [errors] that were the product of seasoned people not sharing information with the [new nurses]. [In one in particular], the nurse needed to give a heparin bolus to the patient and she brought the vial to the preceptor and said, "This seems a little funny to me because..." She was reading the vial wrong, so instead of giving 5000 units, she gave 50,000 units of heparin, ...So this idea of, you know, not sharing information is a violent act as far as I'm concerned, and not only what it does to patient care, but young people coming into our profession end up leaving because they can't

tolerate the way they're being treated at work. (NMF14)

We've hired 27 new nurses in the last six months because of the negativity that goes on ... then we have to hire people that don't have critical care experience, don't have ER experience, but yet we have all these holes, so "Let's cut your orientation short," so now we have new nurses that have no ER experience that are getting four weeks of orientation ... absolutely, it affects the quality of the patient care. (SNF16)

4.5. Positional maps (actor response): the positions taken (and not taken) by each group

The final component of our model describes responses to bullying in three primary manifestations: acting as the *guilty bystander*, *maintaining the status quo*, and *calling it out*. While research suggests that individual manager interventions are more effective than other approaches [31], participants spoke to the difficulty of managers dealing directly with bullying behaviors.

... every one of us says, "We know who they are, we know who they are," and... as a leader, it's hard to address because a lot of times, for me, when I say, "Hey, I understand that you're telling me this; will you put it in writing?" and nobody will ever put it in writing. The only time they'll finally put it in writing is when like 10 people put it in writing because everyone is just done. But, it's that little behavior that, if we don't call out the little behavior, the little behavior becomes big behavior and it just gets worse and worse and worse. (NMM2)

The guilty bystander is unwilling to call out the behavior for fear of attracting unwanted attention for naming and addressing bullying behaviors; silence and inaction by guilty bystanders can impede efforts to resolve bullying behaviors [32]. This element of our model is consistent with a model developed by Australian researchers [33], describing the influence of informal workplace alliances on bullying.

I'll say, "Well, can you write that up?" Well, they don't want their name on anything because they have a good relationship with this person, so they don't want a relationship that's not strong and is not good with those physicians that they're working with. (NMF7)

I went to management and like, "these are the patients she's sending to inappropriate areas, A, B, C, and D. What are you going to do about it? And when that didn't happen from a manager, we just shook our head and said, "I'm done." Why am I going to continue going up the [chain of command] ladder if I'm getting no results? (SNF25)

Another aspect of maintaining the status quo may be related to managing the emotional exhaustion that is a corollary of workplace bullying [18].

Now I just put my head down and make sure my patients are taken care of because there have been enough times I've done that and then it ends up coming back to... I feel like if I get back where then they start focusing their bullying at me. (SNF16)

The final actor response, calling it out, is a strategy that is particularly effective [34], but also requires the most collaboration between staff and management.

... this director that I have has really changed the atmosphere in our emergency room because it's zero tolerance. There is no bullying; you will go into her office; you will address the fact that you spoke to this person this way and you did this, you know. And then, the best thing that comes out of that: "Oh I didn't realize I was being that way." They become, you know, co-workers; they become a part of the team. (SNF33)

... when she [the last bully] walked out of that door at 7:15 that night, the ER was a totally different place. The tone, the people, people volunteering for overtime, I can't tell you. People came up to me that I had no idea were affected by her and some of the things that were going on, the things taking place, she's doing, and she was a charge nurse. (NMF1)

Team building initiatives, coaching, mentorship, and preceptor training were mentioned as interventions to reduce bullying behavior. Other components included regular rotation for all assignments (to avoid favoritism or punitive assignments) and implementation of preventive policies such as codes of conduct, zero tolerance, and social media guidelines. These recommendations support results from other studies that suggest a collaborative management style creates environments that are more resistant to bullying [6].

5. Conclusions and implications

Workplace bullying in the emergency care setting is a significant factor in the dynamics of patient care, nursing work culture, and nurse retention. Specifically, the effect on patient care is significant, with both direct (errors and substandard care) and indirect (high turnover and inexperienced staff hired to replace those RNs who have left to escape bullying behavior) sequelae.

The theory describes the actors' responses to bullying as it occurs, how each response affects the frequency of bullying behaviors, and the influence the dominant narrative and environmental elements might have on the prevalence of bullying. Our model suggests that where guilty bystanding, maintaining the status quo, or retaliation responses are pronounced, bullying will be more pronounced than in EDs where the calling-it-out strategy is employed.

An assessment of hospital work environments should include nurse perceptions of and responses to workplace bullying. Future research should include testing of the theoretical coherence of the model, and testing of interventions to determine the effect on workplace bullying incidents, nurse retention, and patient outcomes.

6. Limitations

Participants in this study were self-selected and, although representative of the general population of emergency nurses in the United States, their perceptions may not reflect the general population of nurses working in emergency care.

Conflict of interest

None.

Ethical statement

Reviewed by Chesapeake IRB and determined to be exempt from IRB oversight.

Funding

This research did not receive any specific grant from funding agencies in the public, commercial, or not-for-profit sectors.

Acknowledgements

The authors wish to acknowledge Leslie Gates for her assistance with this study, and the members of the Institute for Emergency Nursing Research Advisory Council (Hershaw Davis, Jr., MSN, RN, Nancy Knechel, PhD, MSN, BS, RN, ACNP-BC, APRN-BC, and Anita Smith PhD, RN, CNS) for their review of this manuscript.

References

- [1] Page A, Institute of Medicine (US), Committee on the Work Environment for Nurses and Patient Safety. Keeping patients safe: transforming the work environment of nurses; 2004.
- [2] Clarke AE. Situational analyses: grounded theory mapping after the postmodern turn. *Symbol Interact* 2003;26(4):553–76.
- [3] Scott D, Usher R. Researching education: data, methods and theory in educational enquiry. London, New York: Continuum International Pub. Group.; 2011.
- [4] Usher R. Telling a story about research and research as story-telling: postmodern approaches to social research. *Soc Res Educ Stud Ser* 1997;21(16):27–41.
- [5] Einarsen S, Skogstad A. Bullying at work: epidemiological findings in public and private organizations. *Eur J Work Org Psychol* 1996;5(2):185–201. <http://dx.doi.org/10.1080/13594329608414854>.
- [6] Spector PE, Zhou ZE, Che XX. Nurse exposure to physical and nonphysical violence, bullying, and sexual harassment: a quantitative review. *Int J Nurs Stud* 2014;51(1):72–84. <http://dx.doi.org/10.1016/j.ijnurstu.2013.01.010>.
- [7] Wilson BL, Diedrich A, Phelps CL, Choi M. Bullies at work: the impact of horizontal hostility in the hospital setting and intent to leave. *J Nurs Adm* 2011;41(11):453–8. <http://dx.doi.org/10.1097/NNA.0b013e3182346e90>.
- [8] Vogelpohl DA, Rice SK, Edwards ME, Bork CE. New graduate nurses' perception of the workplace: have they experienced bullying? *J Prof Nurs* 2013;29(6):414–22. <http://dx.doi.org/10.1016/j.profnurs.2012.10.008>.
- [9] Bowles C, Candela L. First job experiences of recent RN graduates: improving the work environment. *J Nurs Adm* 2005;35(3):130–7.
- [10] Verkuil B, Atasayi S, Molendijk ML. Workplace bullying and mental health: a meta-analysis on cross-sectional and longitudinal data. *PLoS One* 2015;10(8):e0135225. <http://dx.doi.org/10.1371/journal.pone.0135225>.
- [11] Trépanier SG, Fernet C, Austin S, Boudrias V. Work environment antecedents of bullying: a review and integrative model applied to registered nurses. *Int J Nurs Stud* 2016;31(55):85–97. <http://dx.doi.org/10.1016/j.ijnurstu.2015.10.001>.
- [12] Bowling NA, Beehr TA. Workplace harassment from the victim's perspective: a theoretical model and meta-analysis; 2006 998. Abstract.
- [13] Laschinger HK, Wong C, Regan S, Young-Ritchie C, Bushell P. Workplace incivility and new graduate nurses' mental health: the protective role of resiliency. *J Nurs Adm* 2013;43(7/8):415–21. <http://dx.doi.org/10.1097/NNA.0b013e31829d61c6>.
- [14] Nielsen MB, Einarsen S. Outcomes of exposure to workplace bullying: a meta-analytic review. *Work Stress* 2012;26(4):309–32. <http://dx.doi.org/10.1080/02678373.2012.734709>.
- [15] Samnani AK, Singh P. 20 years of workplace bullying research: a review of the antecedents and consequences of bullying in the workplace. *Aggress Violent Behav* 2012;17(6):581–9. <http://dx.doi.org/10.1016/j.avb.2012.08.004>.
- [16] Edward KL, Ousey K, Warelou P, Lui S. Nursing and aggression in the workplace: a systematic review. *Br J Nurs* 2014;23(12):653–9. <http://dx.doi.org/10.1080/13594329608414854>.
- [17] Lindy C, Schaefer F. Negative workplace behaviours: an ethical dilemma for nurse managers. *J Nurs Manag* 2010;18(3):285–92. <http://dx.doi.org/10.1111/j.1365-2834.2010.01080.x>.
- [18] Wolf LA, Perhats C, Delao AM, Clark PR. Workplace aggression as cause and effect: Emergency nurses' experiences of working fatigued. *Int Emerg Nurs* 2016. <http://dx.doi.org/10.1016/j.ienj.2016.10.006>. Advance online publication.
- [19] Dittmer D. A safe environment for nurses and patients: halting horizontal violence. *J Nurs Regul* 2010;1(3):9–14.
- [20] Sellers KF, Millenbach L, Ward K, Scribani M. The degree of horizontal violence in RNs practicing in New York State. *J Nurs Adm* 2012;42(10):483–7. <http://dx.doi.org/10.1097/NNA.0b013e31826a208f>.
- [21] Townsend T. Break the bullying cycle. *Am Nurse Today* 2012;7(1):12–5.
- [22] Blando J, Ridenour M, Hartley D, Casteel C. Barriers to effective implementation of programs for the prevention of workplace violence in hospitals. *Online J Issues Nurs* 2015;20(1).
- [23] Vessey JA, DeMarco R, DiFazio R. Bullying, harassment and horizontal violence in the nursing workforce. *Annu Rev Nurs Res* 2011;28(1):133–57. <http://dx.doi.org/10.1891/0739-6686.28.133>.
- [24] Alert SE. Behaviors that undermine a culture of safety. Sentinel event alert; 2008;9(40). Retrieved from < https://www.jointcommission.org/assets/1/18/SEA_40.PDF > .
- [25] Bowie V. Defining violence at work: a new typology; 2002.
- [26] Meissner JE. Nurses are we still eating our young? *Nursing* 1999;29(2):42–9.
- [27] Trépanier SG, Fernet C, Austin S. Workplace psychological harassment in Canadian nurses: a descriptive study. *J Health Psychol* 2013;18(3):383–96. <http://dx.doi.org/10.1177/1359105312443401>.
- [28] Carter M, Thompson N, Crampton P, Morrow G, Burford B, Gray C, Illing J. Workplace bullying in the UK NHS: a questionnaire and interview study on prevalence, impact and barriers to reporting. *BMJ Open* 2013;3(6):e002628.
- [29] Dellasega CA. Bullying among nurses. *Am J Nurs* 2009;109(1):52–8.
- [30] Hutchinson M, Vickers MH, Wilkes L, Jackson D. A typology of bullying behaviours: the experiences of Australian nurses. *J Clin Nurs* 2010;19(15–16):2319–28. <http://dx.doi.org/10.1111/j.1365-2702.2009.03160.x>.
- [31] Skarbek AJ, Johnson S, Dawson CM. A phenomenological study of nurse manager interventions related to workplace bullying. *J Nurs Adm* 2015;45(10):492–7. <http://dx.doi.org/10.1097/NNA.0000000000000240>.
- [32] Fitzgerald EM, Myers JG, Clark P. Nurses need not be guilty bystanders: caring for vulnerable immigrant populations. *OJIN* 2017;22(1). <http://dx.doi.org/10.3912/OJIN.Vol22No01PPT43>.
- [33] Hutchinson M, Wilkes L, Jackson D, Vickers MH. Integrating individual, work group and organizational factors: testing a multidimensional model of bullying in the nursing workplace. *J Nurs Manag* 2010;18(2):173–81. <http://dx.doi.org/10.1111/j.1365-2834.2009.01035.x>.
- [34] Laschinger HK, Fida R. A time-lagged analysis of the effect of authentic leadership on workplace bullying, burnout, and occupational turnover intentions. *Eur J Work Org Psychol* 2014;23(5):739–53. <http://dx.doi.org/10.1080/1359432X.2013.804646>.